Documentation Tips

General Education

The Advisory Board Company

Documentation Overview

Your documentation tells a patient's story.

- H&P = Introduction
- Progress/Op Notes = Body
- Discharge summary = Conclusion

It is critical to paint a clear picture from start to finish and cover the initial situation, changes through the stay, and a clear summary that brings it all together.

Documentation Best Practices

- Always document the diagnosis(es) that contributed to the reason for inpatient admission
- Indicate what additional diagnoses are present on admission (POA)
- Indicate acuity/severity of all diagnoses: acute, chronic, acute on chronic, exacerbation
- Indicate "suspected," "probable," "can't rule out," "presumed" or "likely" if you are treating a condition based on medical decision making or risk factors (e.g. probable sepsis due to acute cholecystitis). Coding guidelines require that uncertain diagnoses are reconfirmed at the time of discharge (include in the discharge summary).
- Identify the significance of radiology/pathology/diagnostic test results including lab with a corresponding diagnosis in your documentation
- Link each diagnosis to signs/symptoms/clinical indicators/descriptors (e.g.; "altered mental status secondary to metabolic encephalopathy") and treatment
- Link diagnoses to underlying etiology or manifestations whenever possible (e.g. "GI bleed due to peptic ulcer hemorrhage" or DM2 uncontrolled with PVD and ischemic ulcer of left calf)
- Clearly identify conditions that have been ruled out and those resolved.
- Utilize consults to improve specificity of diagnoses, if agree, then confirm, The attending needs to clarify conflicting documentation by various providers within the record. Instead of "appreciate hospitalist's consult", can say "hospitalist's consult reviewed and agreed.
- Avoid use of arrows/symbols. Spell out acronyms and abbreviations at least one time (e.g., hyponatremia instead of ↓Na)

Key General Education Documentation Requirements in ICD-10-CM

ICD-10-CM codes require additional specificity for code assignment. To reduce coder queries and ensure coded data properly capture conditions treated, provide the following additional specificity. **Unspecified diagnoses do not exist for some conditions in ICD-10-CM.**

Key ICD-10-CM Documentation Concepts	
Specific anatomical location	**Degree** (mild, moderate, severe, or unspecified; total/complete vs. partial/incomplete)
Type (primary, secondary, unspecified)	**Episode of Care** (Initial, Subsequent, Sequelae)
Acuity (acute, subacute, chronic, acute on chronic, or unspecified)	**Laterality** (Right, Left, bilateral, or unspecified)
Trimester (1,2,3,unspecified)	**Number of fetus** (1-5, other)

Present On Admission

Present On Admission (POA) indicators must be submitted for all diagnoses on claims involving inpatient admissions to acute care hospitals.

- ✓ There is **no required timeframe** as to when the provider should identify or document a condition as POA.
- ✓ POA codes assignment directly impact quality reporting

Documentation Examples:
- UTI due to indwelling Foley, POA
- MRSA infection due to central venous catheter, POA

Encounter Type for Injuries and Poisonings	
Initial Encounter	Acute care or first time seen by the physician
Subsequent Encounter	Patient is receiving routine care during healing or recovery phase
Sequelae	Visit for a sequelae or late effect of a previous problem *(Not used for the initial acute problem)*

*High Impact Diagnosis **Low Impact Diagnosis

Capturing Severity of Illness (SOI) in ICD-10-CM Terms

A patient's SOI is conveyed to CMS and quality organizations via ICD-10-CM codes, assigned by a coder reading the medical record. Document known or suspected relationships between concomitant conditions wherever possible to ensure accurate capture of the patient's true risk of mortality and/or readmission.

If Observing...	Please Consider Documenting... (Higher SOI)	
Chronic Kidney Disease	Identify the Stage: I-V Stages IV & V**	
Mass Effect or Midline Shift	Clarify significance with terms such as: • Cerebral edema* • Vasogenic edema* • Brain herniation* • Brain compression* • Insignificant radiology finding	
Altered Mental Status	• Encephalopathy – Type: hepatic, metabolic*, hypertensive**, septic*, toxic*, post operative – Acuity: acute* or chronic • Urinary Tract Infection • Delirium with associated condition and acuity • Acute confusion**	
BMI ≥40 or BMI <19****	Provider must document the correlating medical diagnoses: • Morbid obesity • Obesity • Cachexia**	
Pressure Ulcers **Note:** Non pressure ulcers also need depth (tissue level) documented	Include : • Type (pressure) • Laterality • Site • Stage 1-4 or unstageable (Stages 3 & 4 *)	
	Provider also must document whether it is Present On Admission (POA)	
Diabetes Mellitus	Type	• Diabetes Type 1 • Diabetes Type 2
	Specify if **due to** underlying condition	• (e.g. drug or chemical induced, cystic fibrosis, malignant neoplasm of pancreas, adverse effect of drug)
	Identify the *cause and effect relationship* between the manifestation and the DM before it can be coded to a DM complications (e.g.. CKD Stage 4 due to DM Type 2)	
	Note: If the type of DM is not documented or unclear the default is Type 2	

Respiratory Failure	
Acuity	• Acute* • Acute on Chronic* • Chronic** • Acute **postprocedural** (J95.821)* • Acute on chronic **postprocedural** (J95.822)*
Specificity	With • Hypoxia* • Hypercapnia* • Unspecified*

Sepsis		
Sepsis*	Identify Organism	Document what Sepsis is ***due to***: e.g. Streptococcus (Group A or B), Staphylococcus aureus, MSSA, MSRA, Hemophilus influenzae, anaerobes. Gram-negative organism, Escherichia Coli, Serratia, Enterococcus
Severe Sepsis*	Acuity	"Severe"
	Identify Organism	See above
	Document with or without organ dysfunction • With acute organ dysfunction • With multiple organ dysfunction • SIRS d/t infectious process with acute organ dysfunction	
	Document with or without Septic Shock*	

Note: The term "Urosepsis" is not considered synonymous with sepsis.. It does not default to UTI in ICD-10-CM. Should a provider use this term a query must be submitted for clarification

SIRS Criteria	
Temperature	< 96.8°F(36°C) or > 100.4°F (38°C)
Heart Rate	> 90 bpm
Respiratory Rate	> 20 breaths/min **or** $PaCO_2$ < 32 mmHg
White Blood Cell Count	> 12,000 or < 4,000 cells/mm^3 **or** > 10% bands

GI Hemorrhage	
Acuity	• Acute or Chronic
Link to underlying condition **Note: Active bleeding does not have to be present**	• Gastritis • Hemorrhage of anus and rectum • Angiodysplasia of stomach with hemorrhage • Diverticular disease with hemorrhage • Gastritis and duodenitis with hemorrhage • Peptic ulcer with hemorrhage

Documentation Tips

- Anemia-specify the acuity: acute blood loss, chronic blood loss or anemia 2° to CKD.
- *Stage the CKD:*

  Stage I- Kidney damage with normal kidney function- GFR >90

  Stage II- Kidney damage with mildly decreased kidney function- GFR 60-90

  Stage III-Kidney damage with moderate decreased kidney function- GFR 30-59

  Stage IV-Kidney damage with severely decreased kidney function- GFR 15-29

  Stage V- Kidney Failure- GFR <15

  ESRD- End Stage Renal Disease
- Pneumonia; Link the causative organism and confirm the cause if known I.E. aspiration.
- CHF**-** Document type - diastolic/systolic/combined. Document acuity- acute/chronic/acute on chronic.

- Acute renal insufficiency does not equal acute renal failure.
- Working/Interim diagnosis must be confirmed or ruled out at time of discharge.
- Possible or probable diagnosis cannot be coded on out-patient or observation charts.
- Transbronchial biopsy is not a codeable procedure. It must be documented as a lung or bronchial biopsy.
- Urosepsis is not a codeable diagnosis. It must be documented as Sepsis OR UTI.
- Organisms must be linked with the diagnosis I.E. sepsis 2° to E-Coli, UTI 2° to E-Coli.
- Debridement-excisional or non-excisional and document the level of tissue removed I.E.. skin/subcutaneous tissue, fascia, muscle or bone
- UTI and catheter are present- clarify cause/effect relationship with the infection and catheter
- *Intracerebral bleed, hemorrhage or hematoma- specify traumatic or non-traumatic*

MS-DRGs (Medical)

872 Sepsis w/o MCC: GMLOS 3.50 Days
871 Sepsis w/MCC: GMLOS 4.80 Days

293 Acute CHF w/o CC/MCC: GMLOS 2.30 Days
292 Acute CHF w/CC: GMLOS 3.10 Days
291 Acute CHF w/MCC: GMLOS 4.00 Days

179 Respiratory Infections (Complex Pneumonia/COVID19) w/o CC/MCC: GMLOS: 3.00 Days
178 Respiratory Infections (Complex Pneumonia/COVID19) w/ CC: GMLOS 4.00 Days
177 Respiratory Infections (Complex Pneumonia/COVID19) w/MCC: GMLOS 5.40 Days

195 Simple Pneumonia/Influenza w/o CC/MCC: GMLOS 2.50 Days
194 Simple Pneumonia/Influenza w/CC: GMLOS 3.10 Days
193 Simple Pneumonia/Influenza w/MCC: GMLOS 4.10 Days

690 Acute UTI w/o MCC: GMLOS 2.90 Days
689 Acute UTI w/MCC: GMLOS 3.80 Days

379 Acute GI Bleed w/o CC/MCC: GMLOS 2.10 Days
378 Acute GI Bleed w/CC: GMLOS: 3.00 Days
377 Acute GI Bleed w/MCC: GMLOS 4.40 Days

392 Abdominal Pain unspecified/Acute Diverticulitis w/o MCC: GMLOS 2.60 Days
391 Abdominal Pain unspecified/Acute Diverticulitis w/MCC: GMLOS 3.70 Days

192 COPD Exacerbation w/o CC/MCC: GMLOS 2.40 Days
191 COPD Exacerbation w/CC: GMLOS 2.90 Days
190 COPD Exacerbation w/MCC: GMLOS 3.60 Days

603 Cellulitis w/o MCC: GMLOS 3.20 Days
602 Cellulitis w/MCC: GMLOS 4.60 Days

MS-DRGs (Medical)

684 Acute Renal Failure w/o CC/MCC: GMLOS 2.20 Days
683 Acute Renal Failure w/CC: GMLOS 3.10 Days
682 Acute Renal Failure w/MCC: GMLOS 4.30 Days

641 Metabolic Disturbances (Electrolyte imbalance/Dehydration) w/o MCC: GMLOS 2.60 Days
640 Metabolic Disturbances (Electrolyte imbalance/Dehydration) w/ MCC: GMLOS 3.30 Days

310 Cardiac Arrhythmia/Conduction Disorders w/o CC/MCC: GMLOS 1.90 Days
309 Cardiac Arrhythmia/Conduction Disorders w/ CC: GMLOS 2.40 Days
308 Cardiac Arrhythmia/Conduction Disorders w/MCC: GMLOS 3.50

282 Acute MI w/o CC/MCC: GMLOS 1.80 Days
281 Acute MI w/CC: GMLOS 2.50 Days
280 Acute MI w/MCC: GMLOS 4.10 Days

287 Circulatory Disorders w/o AMI w/Cardiac Cath w/o MCC: GMLOS 2.30 Days
286 Circulatory Disorders w/o AMI w/Cardiac Cath w/MCC: GMLOS 5.30 Days

066 Acute CVA w/o CC/MCC: GMLOS 2.00 Days
065 Acute CVA w/CC: GMLOS 2.90 Days
064 Acute CVA w/MCC: GMLOS 4.40 Days

189 Acute Respiratory Failure/Pulmonary Edema: GMLOS 3.60 Days

069 TIA: GMLOS 2.00 Days

313 Chest Pain: GMLOS 1.70 Days

312 Syncope/Orthostatic Hypotension: GMLOS: 2.30 Days

MS-DRG (Surgical)

470 Total Hip/Knee Procedure w/o MCC: GMLOS 1.80 Days
469 Total Hip/Knee Procedure w/MCC: GMLOS 3.10 Days

482 Hip/Femur Fracture Repair (ORIF) w/o CC/MCC: GMLOS 3.30 Days
481 Hip/Femur Fracture Repair (ORIF) w/CC: GMLOS 4.30 Days
480 Hip/Femur Fracture Repair (ORIF) w/MCC: GMLOS 6.20 Days

468 Revision of Hip/Knee Replacement w/o CC/MCC: GMLOS 1.90 Days
467 Revision of Hip/Knee Replacement w/ CC: GMLOS 3.30 Days
466 Revision of Hip/Knee Replacement w/MCC: GMLOS 7.00 Days

462 Bilateral Hip/Knee Replacement w/o MCC: GMLOS 2.60 Days
461 Bilateral Hip/Knee Replacement w/MCC: GMLOS 6.70 Days

331 Major Small and Large Bowel Procedures w/o CC/MCC: GMLOS 3.30 Days
330 Major Small and Large Bowel Procedures w/CC: GMLOS 5.60 Days
329 Major Small and Large Bowel Procedures w/MCC: GMLOS 10.20 Days

419 Lap Cholecystectomy w/o CC/MCC: GMLOS 2.40 Days
418 Lap Cholecystectomy w/ CC: GMLOS 3.50 Days
417 Lap Cholecystectomy w/MCC: GMLOS 5.10 Days

855 Infectious Disease with OR Procedure w/o CC/MCC: GMLOS 3.30 Days
854 Infectious Disease with OR Procedure w/CC: GMLOS 5.30 Days
853 Infectious Disease with OR Procedures w/MCC: GMLOS 9.60 Days

MS-DRG (Surgical)

247 AMI w/PCI with Drug Eluting Stent w/o MCC: GMLOS 2.10 Days
246 AMI w/PCI with Drug Eluting Stent w/MCC: GMLOS 3.90 Days

244 Permanent Pacemaker Implant Procedure w/o CC/MCC: GMLOS 2.20 Days
243 Permanent Pacemaker Implant Procedure w/CC: GMLOS 3.10 Days
242 Permanent Pacemaker Implant Procedure w/MCC: GMLOS 5.20 Days

236 CABG Procedures w/o Cardiac Catheterization w/o MCC: GMLOS 6.00 Days
235 CABG Procedures w/o Cardiac Catheterization w/MCC: GMLOS 8.60 Days

221 Cardiac Valve Procedures w/o AMI w/o Cardiac Cath w/o CC/MCC: GMLOS 3.90 Days
220 Cardiac Valve Procedures w/o AMI w/o Cardiac Cath w/CC: GMLOS 5.90 Days
219 Cardiac Valve Procedures w/o AMI w/o Cardiac Cath w/MCC: GMLOS 8.90 Days

267 TAVR Procedure w/o MCC: GMLOS 1.70 Days
266 TAVR Procedure w/MCC: GMLOS 3.20 Days